

Simple Prostatectomy

WARWIKI

Patient Information · Surgery for a very large enlarged prostate (BPH)

Simple prostatectomy is surgery for a **very large prostate** that blocks the flow of urine. The surgeon removes the bulky inner part of the prostate, which causes the blockage. It is done through the belly, either with one larger cut (open surgery) or with small cuts and a robot. It works very well for big prostates.

An Important Difference

"Simple" prostatectomy is **not** the "radical" operation for prostate **cancer**. A radical operation removes the whole prostate. A simple prostatectomy removes only the enlarged inner tissue, which is not cancer, and leaves the outer shell.

Benefits and Risks in Numbers

9 in 10

men do not need another prostate operation within 5 years

8–9 in 10

have little or no semen afterward (harmless)

7–14 in 100

need a blood transfusion (fewer with robotic surgery)

Numbers are averages from published studies. Your own risk may differ; ask your team.

When It Is Used

- For **very large prostates** that are too big for a standard scope surgery (TURP)
- Also when **bladder stones** or large bladder pouches need to be fixed at the same time
- Scope surgeries such as **HoLEP** or **Aquablation** are other choices for large prostates. Your surgeon will help you choose.

Learn the Terms

Simple prostatectomy

Removing the enlarged inner prostate for BPH. It is not a cancer operation.

Adenoma

The enlarged inner prostate tissue that is removed. It is not cancer.

Radical prostatectomy

A different operation that removes the whole prostate for cancer.

Open or robotic

Surgery through one larger cut, or through several small cuts with a robot.

Retrograde ejaculation

Semen goes back into the bladder instead of out.

Catheter

A soft tube that drains the bladder for several days after surgery.

Will it hurt? You are fully asleep, so you feel nothing during surgery. Afterward you will have some belly soreness. Recovery takes **longer than after a scope surgery**. The hospital stay ranges from one day to several days, and is often shorter with robotic surgery.

In the hospital

Catheter, and sometimes a drain. Some men go home within a day.

First week or so

The catheter stays several days to about a week.

First few weeks

Pink urine at first. No heavy lifting or straining.

Follow-up visit

Your team checks your flow and healing.

How to Get Ready

- Follow your instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection first.
- Stop smoking. Arrange a ride and help at home.

What Happens

- 1 You get **general anesthesia**, so you are fully asleep, and antibiotics.
- 2 Through open or robotic surgery, the inner prostate is removed. The outer shell stays.
- 3 A catheter, and sometimes a drain, is placed.

What to Expect After

- A strong urine stream that lasts.
- Leakage when you cough or lift can happen early and usually gets better. It lasts in 3–8 in 100 men.
- New erection problems are uncommon.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- A cut that is red, swollen or leaking, or belly pain that gets worse

Go to an emergency department if:

- You cannot urinate for several hours, or the catheter stops draining
- Your urine looks like red wine or has clots
- You have chest pain, trouble breathing, or a swollen, painful leg
- You cannot reach the office and are worried

Three Things to Remember

1. This surgery removes the inner part of a very large prostate. It is not the cancer operation.
2. It works very well, but recovery is longer than after a scope surgery.
3. Little or no semen afterward is common and harmless.